

Media Information



Baptist Health Medical Center
9601 Baptist Health Drive
Little Rock, Arkansas 72205

Coman, Mihaela M 00543619
F, 70 yrs,
21 BURNTTREE CT, LITTLE ROCK AR
W: M:

Department/Submitter Info:
Baptist Health Breast Center
9500 Kanis Road
Little Rock AR

Addended Report: SURG

Surgical Pathology Report (Final result) S25-

Authorizing Provider:							Health Breast Center
Pathologist:							
Collected:		1436	Received:			1509	

Specimens
A Breast, Right, RIGHT BREAST ARCHITECTURAL DISTORTION @ 9:00

CLINICAL INFORMATION
Right breast architectural distortion

ABNORMAL/SIGNIFICANT FINDINGS (Abnormal)
Yes Ref. Range: No

FINAL DIAGNOSIS
A. Breast, Right Architectural Distortion, Core Biopsy:
- Invasive ductal carcinoma, Nottingham grade 1 of 3, 0.2 cm in this material; see comment.

Electronically signed by Melody Harrison, MD on at 1704 CDT

DIAGNOSIS COMMENT
A p63 immunostain demonstrates loss of myoepithelial cells in the areas of interest, supporting the above diagnosis. The Nottingham grade is based on scores of 2 for architecture, 2 for cytology, and 1 for mitotic activity. Ancillary studies for ER, PR, HER2, and Ki-67 are in progress, and the results will be issued in a supplemental report.

GROSS DESCRIPTION

**Baptist Health****Baptist Health Medical Center
9601 Baptist Health Drive
Little Rock, Arkansas****A. RIGHT BREAST ARCHITECTURAL DISTORTION @ 9:00**

Received in 10% neutral buffered formalin in a container labeled with the patient's name [REDACTED] architectural distortion at 9:00 are multiple fragmented pieces of fibrofatty tissue that measure 2.7 cm in aggregate. This entire specimen is submitted.

(Multiple sections, 1 block)

Gross description performed by Savannah Jenkins

**ANCILLARY TESTING
ESTROGEN AND PROGESTERONE RECEPTORS****SOURCE: RIGHT BREAST (blk A1)****Estrogen Receptor IHC: POSITIVE****% of Nuclei Staining (ER): 94%****Staining Intensity: STRONG****Progesterone Receptor IHC: NEGATIVE****% of Nuclei Staining (PR): <1%****Staining Intensity: N/A****INTERPRETATION: See note**

NOTE: Staining of greater than or equal to 1% of tumor cell nuclei is considered positive or favorable and correlates with the presence of ER and PR receptors by Biochemical assay. Staining of less than 1% of tumor cell nuclei is considered negative or unfavorable. Control case and benign tissue in the block stained appropriately.

There are limited data on the overall benefit of endocrine therapies for patients with low level (1-10%) ER expression, but they currently suggest possible benefit; therefore, these patients are considered eligible for endocrine treatment. There are data that suggest invasive cancers with these results are heterogeneous in both behavior and biology and often have gene expression profiles more similar to ER-negative cancers. The "Low Positive" designation only applies to invasive carcinoma.

HER-2/NEU**RESULT: NEGATIVE****Staining Intensity: 1+****INTERPRETATION: See note**

NOTE: This test is performed by immunohistochemical methodology. HER2/neu staining intensity of 3+ is considered

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unequivocally positive. Staining intensity of 2+ is considered equivocal, therefore in situ hybridization (ISH) analysis will be reflexively ordered by our laboratory and the results will be given in an addendum report. HER2/neu staining of 0 and 1+ is considered negative. If HER2/neu ISH is required on cases other than those with 2+ staining, please notify this office. Control case tissue and benign tissue in the block stain appropriately.

Ki-67

RESULT: FAVORABLE

Percent Positive: 5.6%

INTERPRETATION: See note

NOTE: This test is performed by immunohistochemical methodology. Staining of less than 10% of tumor cell nuclei is considered negative or favorable. Staining of 10% to less than 20% of tumor cell nuclei is considered borderline. Staining of greater than or equal to 20% of tumor cell nuclei is unfavorable.

This ER, PR, and Ki67 test was developed by Pathology Labs of Arkansas has been approved by the U.S. Food and Drug Administration. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

These assays have not been validated on decalcified specimens or tissue other than breast. Nor is it FDA approved for these specimens.

This test panel was analyzed using the Ventana Image Analysis System (DP200). Type of fixation: 10% neutral buffered formalin. Embedded in paraffin. Tissue is fixed in 10% NBF for 6 to 72 hours. As in the ASCO/CAP HER2 guideline, storage of slides for more than 6 weeks before analysis is not recommended. Immunohistochemical stains were performed on Ventana automated analyzers. Ventana Ultraview DAB detection system. Antibodies: Anti ER clone SP1 Ventana, Anti PR clone 1E2 Ventana, Anti Ki 67 clone K-2 Ventana, Anti Her-2/neu clone 4B5 Ventana.

Addendum electronically signed by Melody Harrison, MD on [REDACTED] at 1310 CDT

DISCLAIMER

Pathology report signed out by Pathology Laboratories of Arkansas at:
 Baptist Health Medical Center - Little Rock
 9601 Baptist Health Dr., Little Rock, AR [REDACTED]

NOTE: Corresponding control slides are noted to stain appropriately for immunohistochemistry stain(s) and/or special stain(s), if applicable.

Immunohistochemical staining may involve reagents labeled as analyte specific reagents (ASR). These assays were developed and their performance characteristics were determined by the Histology Laboratory at Pathology

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Laboratories of Arkansas. They have not been cleared or approved by the FDA. The FDA has determined that such clearance or approval is not necessary. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 as qualified to perform high complexity clinical laboratory testing.

Resulting Labs

PLA	PATHOLOGY LABORATORIES OF ARKANSAS, #1 Lile Ct., Suite 101, Little Rock AR Director: Palmer, Hal Eugene, MD
	CLIA:

Legend

A - Abnormal

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Document Information

Other: Outside Correspondence

00:00

Attached To:
Michaela Maria Coman

Source Information
Mahnoor Siddiqui | Baic Bre Srg
